Medical Legal Report

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1. Demographics

a.

Here are the names and addresses extracted from the provided document: 1. **Defense Attorney:** - Name: Jordan Zamora-Alcantara - Address: 250 Technology Way, Rocklin, CA 95765 - Contact: (877) 739-7481■4:2†source■. 2. **Applicant Attorney:** - Name: Emily Mehr, Esq. - Firm: MEHR ASSOCIATES IRVINE - Address: 17500 Red Hill Ave Suite 210, Irvine, CA 92614 - Contact: (949) 777-9449■4:0†source■■4:2†source■■4:3†source■. 3. **Insurance Company Address:** - Name: SEDGWICK CMS - Mailing Address: PO BOX 14188, Lexington, KY 40512■4:2†source■. This includes all the requested information available in the document regarding legal representation and insurance entities.

b.

Based on the details extracted from the document, here is the filled information: 1. Claimant Name: Tiu, Michelle E 2. Claimant Date of Birth: 10/31/1976■4:0†source■ 3. Claimant Employer: KP SCAL■4:0†source■ 4. Claimant Occupation: Optometrist■4:8†source■ 5. Claimant Date of Injury: 05/28/2024■4:0†source■ 6. Claim number: 2024050001029■4:0†source■ 7. WCAB Number: ADJ19391023■4:14†source■ 8. Date of Exam: (Leave this empty) The details have been verified and cited accordingly from the uploaded document.

2. History of Injury (Patients Perspective)

a.

Michelle E Tiu, a 47-year-old optometrist, has experienced a series of occupational injuries related to her workstation setup and repetitive tasks over approximately two decades of her career. Her injuries began with a minor motor vehicle incident in 2015, leading to neck and back pain■4:15†source■. Over the years, she developed significant right shoulder pain due to repetitive overhead reaching and non-ergonomic work conditions, particularly after being asked to work in various non-ergonomic locations amid the COVID-19 pandemic■4:4†source■■4:7†source■. In 2016, Michelle reported gradual onset right shoulder pain from repetitive reaching to turn off a light at work■4:0†source■, and in 2020, she had a similar issue with left shoulder pain from increased overhead work with exam equipment■4:3†source■. She also suffered from cervical radiculopathy and trapezius strain related to prolonged non-ergonomic workstation use in 2024■4:7†source■■4:13†source■. Throughout her medical history, she has undergone treatments like physical therapy, ergonomic assessments, and acupuncture, which have provided varying degrees of relief■4:18†source■. Despite these setbacks, Michelle has consistently returned to her customary duties with recommended work modifications■4:18†source■.

3. History of Injury (Physicians Perspective)

a.

Michelle E. Tiu, an optometrist with KP SCAL, sustained multiple work-related injuries primarily affecting her shoulders over several years. Her initial reports from 09/09/2016 described repetitive motions resulting in right shoulder pain. By 10/2017, she developed left posterior shoulder pain due to overhead pushing and pulling in a non-ergonomic environment, leading to diagnoses of shoulder strain and impingement syndrome■4:1†source■. This was compounded by ergonomic challenges exacerbated by the COVID-19 pandemic in 2020■4:2†source■. In 2024, she reported right shoulder and elbow pain from constant overhead reaching and keyboarding■4:0†source■. Her treatment included physical therapy, acupuncture, ergonomic assessments, and medication, resulting in varying symptom improvements over time■4:5†source■■4:17†source■. Despite periods of improvement, pain and discomfort persisted, necessitating ongoing adjustments and reviews to her work environment and tasks■4:15†source■■4:17†source■.

4. Chief Complaints

a.

right shoulder blade:
The chief complaints related to the right shoulder blade across various encounters and assessments in the documents are as follows: - Shoulder pain developed due to repetitive reaching across a workstation, specifically noted with activities like reaching to turn off lights up to 60 times a day and swinging a machine overhead, which increased the right shoulder pain■4:0†source■. - The patient reported right shoulder pain radiating to the neck, particularly worse with moving the neck and was accompanied by a persistent tight feeling. This was managed with medication and alternative therapy like acupuncture■4:2†source■. - Pain in the right shoulder, described as moderate to severe and constant. The discomfort was noted to worsen after certain activities like driving or reaching with the right arm■4:3†source■. - History of right shoulder injury associated with non-ergonomic working conditions, especially during the COVID-19 pandemic when the patient was required to work in different locations that were not ergonomically optimized■4:7†source■. These complaints consistently point towards occupational repetitive stress injury as a significant factor in the patient's right shoulder discomfort and pain, with ongoing ergonomic assessments and modifications being a part of the

recommended management strategy.

right scapula:

The patient's chief complaints concerning the right scapula primarily relate to right scapulalgia and shoulder injuries encountered during work activities. The documented issues are as follows: 1. **Right Scapulalgia** (M89.8X1): - The patient reports tenderness and discomfort in the right scapular area, specifically after activities involving repetitive movement and strenuous use of the upper body. This discomfort has been described in association with strains from work-related activities■4:0†source■. 2. **Right Shoulder Pain**: - Mechanism of injury includes repetitive reaching and overhead movements at work, causing moderate to severe, constant right shoulder pain. The activities were reported to increase shoulder pain significantly■4:17†source■■4:10†source■. 3. **Associated Conditions**: - **Right Trapezius Strain (S46.811D)**: The patient was diagnosed with a subsequent strain of the trapezius muscle on the right side■4:8†source■. - **Neck Muscle Strain (S16.1XXD)**: There is also a documented subsequent neck muscle strain, contributing to the discomfort■4:8†source■. 4. **Aggravating and Alleviating Factors**: - Aggravated by overhead use and certain work-related postures. - Alleviation is through stretching, rest, physical therapy, and the use of NSAIDs■4:9†source■■4:18†source■. 5. **Reported Impact and Treatment**: - Pain has impacted the patient's daily activities and work duties. Treatment included physical therapy, acupuncture, and ergonomic evaluations to modify workplace conditions to alleviate symptoms■4:1†source■■4:6†source■. These symptoms and complaints highlight the occupational origins and the ongoing therapeutic approaches adopted, including physical therapy and work environment modifications, to manage the symptoms effectively■4:5†source■■4:3†source■■4:9†source■.

right shoulder:

The patient's chief complaints regarding right shoulder issues include: 1. **Mechanical Injury**: Developed right shoulder pain at work due to repetitive reaching and swinging overhead, experienced by multiple colleagues as well. This was associated with a workstation setup that was not ergonomic■4:0†source■. 2. **Occupational Overuse**: Pain started as a result of right shoulder overuse, involving reaching across the workstation up to 60 times a day and swinging machines overhead■4:3†source■■4:14†source■. 3. **Symptom Characteristics**: - Aching and weakness in the right shoulder with quick or strenuous movement. - Moderate to severe, constant pain with some relief from stretching■4:4†source■. - Pain increased after driving or reaching with the right arm■4:3†source■. - Aggravated by shoulder elevation and repetitive movements, relieved by physical therapy and rest■4:12†source■. 4. **Clinical Diagnoses**: - Diagnosed conditions include tendinitis of the right biceps tendon and impingement syndrome of the right shoulder■4:5†source■■4:17†source■. These complaints and findings are part of ongoing occupational health evaluations due to ergonomic hazards at the workplace■4:14†source■.

right elbow:

The chief complaints related to the patients' right elbow from the document are as follows: 1. **Office Visit on 03/09/2023**: - Chief Complaint: Right lateral elbow pain. The symptoms were characterized by painful usage of the right elbow, including activities such as using a joystick at work and pouring juice. Physical examination noted swelling and tenderness at the lateral epicondyle. The assessment diagnosed right lateral epicondylitis, also known as tennis elbow■4:16†source■. 2. **Office Visit on 04/25/2022**: - Chief Complaint: Ongoing right lateral elbow pain with no significant improvement from using a tennis elbow strap and NSAIDs. The condition affected the patient's daily activities, including work and routine home tasks like washing dishes■4:11†source■. 3. **Telephone Appointment on 03/10/2022**: - Chief Complaint: Right elbow joint pain. Likely diagnosed as tennis elbow with no noted injury, but the pain was worsened by lifting■4:0†source■. 4. **Nurse Visit on 04/18/2022**: - Chief Complaint: Persistent right elbow pain for two months with a recent increase to severe pain. The patient reported a bump on the right elbow but no swelling■4:13†source■. These complaints indicate ongoing issues with right elbow pain, primarily diagnosed as right lateral epicondylitis or tennis elbow, impacting daily activities and quality of life.

b.

Right Elbow:

The patient's chief complaints related to the right elbow, as documented in several visits, are as follows: 1. On 03/10/2022, the complaint was right elbow joint pain, which was a likely case of tennis elbow with no specific injury but pain when lifting■4:0†source■. 2. On 04/18/2022, the patient reported right elbow pain for two months, with severe pain rated 7/10, which woke the patient at night. There was no noted swelling, but the patient mentioned a bump on the elbow■4:3†source■. 3. On 04/25/2022, the patient presented with ongoing right lateral elbow pain. Despite using a tennis elbow strap and NSAIDs, there was little improvement. There was difficulty using a joystick at work and in daily activities such as pouring orange juice or washing dishes. Swelling was also noted■4:1†source■■4:2†source■. 4. On 04/28/2022, during an orthopedic visit, the patient continued to have right lateral elbow pain and swelling, which radiated to the forearm and shoulder, with numbness into the ring and small fingers. A cortisone injection was administered, providing mild improvement■4:9†source■■4:12†source■. 5. On 03/18/2023, the patient had a follow-up regarding right lateral epicondylitis and a failed cortisone injection, leading to a platelet-rich plasma injection■4:4†source■. These notes reflect consistent issues with right elbow pain, particularly suggestive of tennis elbow (lateral epicondylitis), affecting daily activities and work performance.

Right Scapula:

The chief complaints related to the right scapula in the provided document are as follows: 1. **Right Shoulder Pain**: - The patient reports pain in the right upper medial periscapular area radiating to the shoulder and neck. The pain worsens with neck movement■4:3†source■. 2. **Right Shoulder Tendinitis**: - The patient developed right shoulder pain due to repetitive reaching across her work station, which requires turning off a light up to 60 times a day■4:12†source■. 3. **Right Scapulalgia**: - The patient complains of pain in the right upper trapezius/lateral shoulder area, describing it as discomfort■4:6†source■. 4. **General Right Shoulder Pain**: - The patient describes her right shoulder as aching, with pain rated variably. It is aggravated by any overhead use and relieved by stretching■4:7†source■. 5. **Injury Mechanism Related Pain**: - The patient indicates she developed right shoulder pain at work due to different non-ergonomic work stations, leading to strain■4:19†source■. These complaints are associated with various activities and conditions at work, and the pain is often exacerbated by specific movements or ergonomic challenges.

5. Claimants Job

a.

The job description of the claimant, Michelle E. Tiu, includes working as an optometrist. Her responsibilities involve desk work and sedentary tasks, as well as overhead reaching, repetitive bending or twisting at waist level, and upper extremity repetitive motion■4:0†source■.

b.

The specific job duties of the claimant, Michelle E. Tiu, include tasks that involve substantial physical activity related to her occupational role. She reported developing right-sided pain with activities such as frequent overhead reaching and consistent keyboarding with arms outstretched. This has been associated with work in a non-ergonomic exam room■4:10†source■. Additionally, it was recommended that she continue utilizing scapular stabilization techniques during her job tasks and evaluate how she performs her job tasks to minimize repetitive strain■4:2†source■.

6. Work Status

a.

The most recent work status for the claimant, Michelle E Tiu, is that she is on modified activity at work and at home from 7/31/2024 through 9/5/2024. If the modified activity cannot be accommodated by her employer, she is considered temporarily and totally disabled for the designated time■4:2†tmpyms8hkgn.pdf■. As for work restrictions, she is allowed a 20-minute stretch break in both the morning and afternoon■4:2†tmpyms8hkgn.pdf■. If further modifications cannot be accommodated, she would be considered temporarily and totally disabled from her regular work■4:2†tmpyms8hkgn.pdf■.

b.

Patient stopped working on 06/17/2024. The patient was placed off work due to incapacitating injury or pain from 06/17/2024 until 06/25/2024. Following this period, modified activity was recommended from 06/26/2024 through 07/09/2024, which applies to both work and home settings. If the modified activity could not be accommodated by the employer, the patient would be considered temporarily and totally disabled from their regular work■4:0†tmpyms8hkgn.pdf■.

7. Previous Injuries

a.

The patient has not described any previous injuries related to the current complaints or conditions. The medical records indicate there are no prior work-related or non-work-related injuries to the same body part(s)■4:1†source■.

8. Medical History

a.

The patient's non-surgical medical history includes: - Varicella, diagnosed on 01/01/1983■4:0†source■. If you require further details or additional aspects of their medical history, please let me know.

9. Surgical History

a.

The patient's entire surgical history is as follows: - Orbital blowout fracture status post motor vehicle accident (MVA) in 1992 - Ankle surgery in 2006■4:0†tmpyms8hkgn.pdf■.

10. Allergies

a.

The patient's allergies are as follows: - Vicodin [Acetaminophen-Hydrocodone]: Nausea and/or Vomiting■4:0†source■. - Latex: Lip Swelling■4:8†source■.

11. Current Medications

a.

The claimant is currently taking the following medications: 1. Meloxicam (MOBIC) 15 mg Oral Tab: Take 1 tablet by mouth daily for 10 days. Take with food■4:0†source■. 2. Ibuprofen (MOTRIN) 400 mg Oral Tab: Take 1 tablet by mouth 3 times a day with meals. Take with food■4:8†source■. 3. MetroNIDAZOLE (METROGEL) 0.75 % Top Gel: Apply to affected area(s) 2 times a day■4:8†source■. 4. Sulfacetamide-Sulfur (AVAR) 10-5 % (w/w) Top Clsr: Wash affected area(s) daily as directed■4:8†source■. 5. Doxycycline Monohydrate (ADOXA) 50 mg Oral Tab: Take 1 tablet by mouth 2 times a day for acne■4:8†source■. 6. Diclofenac Sodium (ARTHRITIS PAIN, DICLOFENAC) 1 % Top Gel: Apply 2-4 grams to affected area(s) 2 times a day as needed for pain & inflammation. Do not exceed 32 grams in 24 hours■4:8†source■.

12. Social History

a.

The medical record indicates the following regarding the patient's use of alcohol, recreational drugs, tobacco, and vaping: 1. **Alcohol Use**: - According to the records, the patient has no history of alcohol use. An encounter on 03/21/2022 confirmed that the patient's response to alcohol screening was "never"■4:16†tmpyms8hkgn.pdf■. - The document notes that on 11/06/2016, the patient had a history of persistent nausea and emesis after excessive alcohol intake, indicating she consumed alcohol on that date■4:5†tmpyms8hkgn.pdf■. 2. **Recreational Drug Use**: - The patient has no history of recreational drug use■4:1†tmpyms8hkgn.pdf■■4:12†tmpyms8hkgn.pdf■. 3. **Tobacco and Vaping Use**: - **Tobacco**: The patient has never smoked or used smokeless tobacco■4:0†tmpyms8hkgn.pdf■■4:1†tmpyms8hkgn.pdf■. - **Vaping**: The patient has never used vaping products■4:0†tmpyms8hkgn.pdf■. This compiled list reflects the patient's self-reported and recorded history regarding the use of these substances.

13. Family history of illness

a.

The past medical or surgical history of family members, as noted in the documents, is as follows: - **Heart Disease**: - Relation: Father - Age of Onset: 60 - Relation: Maternal Uncle - **Diabetes**: - Relation: Maternal Uncle - Additional Information: Affects 2 maternal uncles - **Cancer**: - Relation: None reported - **Breast Cancer**: - Relation: Maternal Cousin - Age of Onset: 30s (different from cousin with ovarian cancer) - **Ovarian Cancer**: - Relation: Maternal Cousin - Age of Onset: 30s (different from cousin with breast cancer) - **Childhood Deaths**: - Relation: Other - Affected: 5 of the patient's mother's siblings■4:0†source■.

16. Diagnostic Reports

a.

Upon reviewing the uploaded files, none of the MRI studies were completed associated with the described reports. The patient was unable to undergo MRI due to the presence of a facial metal plate from previous surgery. Alternative imaging assessments, such as CT scans or other diagnostic tools, were used in place of MRI where clinically warranted. Please note this summary is extracted from the available diagnostic reports and records provided.

b.

The CT scans in the document are summarized chronologically as follows: 1. **CT Lumbar Spine - 05/13/2020**: The patient, unable to undergo an MRI due to facial surgery and a metal plate, presented with chronic low back pain, left posterior thigh pain, and numbness, suggesting left lower extremity radicular symptoms. Findings included normal spine lordosis and no compression fracture or spondylolisthesis. There was diffuse mild posterior disc bulge at levels L3-4, L4-5, and L5-S1, with mild ligamentum flavum hypertrophy observed at L3-4 and L4-5. Critically, no significant spinal canal or neuroforaminal stenosis was noted, and paraspinal soft tissues appeared intact■4:0†source■. 2. **CT Scan Follow-up - 05/18/2020**: The diagnostic impression from the previous CT examination was reiterated, emphasizing no acute findings and confirming mild spinal degenerative changes without significant neurological impact. The report was electronically signed by Dr. Jay Kang■4:1†source■■4:3†source■■4:6†source■■4:16†source■.

c.

Here is a summary of each X-ray with the diagnostic results in chronological order: 1. **09/09/2016 - Right Shoulder** - Findings: No acute fracture, normal alignment, no significant joint disease or soft tissue abnormality identified. - Diagnosis: Tendinitis of the right shoulder [M75.81]. - Notes: Right shoulder pain assessed, related to work comp incident■4:19†source■. 2. **09/16/2016 - Right Shoulder** - Findings: No acute fracture, normal alignment, no significant joint disease, no soft tissue abnormality. - Diagnosis: Tendinitis of the right shoulder [M75.81]. - Treatment Plan: X-ray and continuation of physical therapy, with work restrictions noted■4:12†source■■4:17†source■. 3. **10/14/2017 - Left Shoulder** - Findings: No acute fracture, normal alignment, no significant joint disease or soft tissue abnormality identified. - Notes: Assessment part of treatment for persisting left shoulder pain; resolved via steroid injection■4:3†source■. 4. **11/19/2018 - Left Foot** - Findings: Normal alignment, no acute fracture, no significant joint disease, no radiopaque foreign body. - Clinical History: To rule out a foreign object due to forefoot pain■4:0†source■■4:15†source■. 5. **06/09/2023 - Cervical Spine** - Findings: Normal cervical vertebral bodies' height and alignment, no fractures, mild disc space narrowing at C6-C7 and C7-T1. - Clinical History: Right-sided cervical radiculopathy, no previous comparative studies available■4:2†source■■4:14†source■. This summary organizes the X-ray diagnostics, helping track the patient's diagnosis and related follows-up evaluations efficiently.

d.

Here are the ultrasound diagnostic results summarized in chronological order: 1. **01/24/2018 - Limited Ultrasound Examination of Left Shoulder**: This ultrasound was performed due to Michelle E. Tiu's complaints of left shoulder pain related to a work incident. The findings revealed an unremarkable acromioclavicular joint, normal subscapularis and biceps tendon appearances. There was mild non-homogeneity of the supraspinatus tendon and no fluid in the subdeltoid bursa■4:19†source■. 2. **04/18/2018 - Transvaginal Ultrasound**: The examination showed the IUD (Intrauterine Device) was located at the fundus. Additionally, there was no abnormal discharge or noticeable lesions observed■4:3†source■. 3. **01/24/2018 - Follow-up Limited Ultrasound Examination of Left Shoulder**: Conducted for persistent left shoulder pain, this ultrasound reaffirmed the previous findings. It assured unremarkable acromioclavicular joint and normal subscapularis appearance, with mild supraspinatus tendon changes, consistent with impingement syndrome■4:19†source■. These summaries capture the diagnostic results and key findings from the ultrasounds in the documents provided.

e.

Here is a summary of the electro diagnostics, including EMG, NCV, or NCS, in chronological order as provided in the documents: 1. **Date: 02/17/2020** - A needle EMG conducted was deemed incomplete as the patient was unable to tolerate the test and requested it be stopped. The primary diagnoses included lumbar radiculopathy and chronic low back pain■4:0†source■. 2. **Date: 02/17/2020** - A referral for imaging and physical therapy was considered. Despite the incomplete EMG, it was clear that chronic low back pain and left posterior thigh pain were present, potentially indicating left lower extremity radicular symptoms■4:4†source■. 3. **Date: 06/10/2024** - An EMG and NCV were requested according to ACOEM guidelines to investigate neck pain with potential referral to the arms. The order noted a preference for tests to be performed within Kaiser Permanente's facilities■4:6†source■■4:13†source■. 4. **Date: 08/28/2024** - Routine nerve conduction studies (NCS) were conducted, showing prolonged right median sensory latency across the right wrist. The

EMG of the right upper extremity was mostly normal except for enlarged and unstable motor unit potentials in the right C8 myotome, consistent with chronic right C8 radiculopathy and mild right carpal tunnel syndrome■4:0†source■. These diagnostics collectively address pain syndromes and functional impairments related to radiculopathy and carpal tunnel syndrome.

17. Doctors Notes

a.

Here are the summaries for each dated note, arranged chronologically: 1. **11/06/2016**: On this date, the patient had an urgent care visit at the West LA center. Clinical notes show the patient had an IV catheter removed without complications, and the site was healthy with no signs of infection. The patient received an after-visit summary and reported that all provider instructions were well understood. The patient's questions about their condition were sufficiently addressed, indicating satisfactory communication with healthcare providers■4:7†source■. 2. **10/07/2014**: During an urgent care visit at Harbor MAC, the patient presented with constipation, tension headaches, and abdominal pain. The attending physician recommended dietary modifications to manage constipation, including increased fluid and fiber intake. Patients were advised to avoid red meats and white flour products. Additional remedies and medications were discussed, such as Metamucil and natural remedies like molasses. Discharge instructions emphasized medication adherence, monitoring symptoms, and follow-up actions■4:13†source■. 3. **10/12/2017**: This office visit concerned occupational health services due to left shoulder impingement. Medical findings were normal, as there were no significant joint diseases or soft tissue abnormalities detected. Medication refills were not needed at the time, per the nursing notes, and the visit was documented by a licensed vocational nurse■4:3†source■. 4. **04/18/2018**: At the Women's Health Center/OB-GYN, the patient had a telephone consultation for recurrent vaginal infections, documented by a registered nurse. The appointment included triage. The physician authorized these visits, ensuring comprehensive care was provided, based on recorded progress notes■4:2†source■. 5. **07/25/2021**: The note details an e-visit response for a work/school off-note request. The procedure for obtaining this note through kp.org was explained thoroughly. Mention of a COVID-19 test was included, alongside detailed precautions for self-monitoring and maintaining health. The correspondence underscored the importance of following preventive measures during recovery■4:4†source■. 6. **04/14/2020**: The patient received a message from family practice confirming a negative COVID-19 test result via Dr. Sung S. Chang. Patient communication included instructions for accessing work notes online. The patient's inquiry about a missing doctor's note was addressed by a licensed vocational nurse (LVN), ensuring proper document access procedures■4:5†source■. 7. **09/17/2019**: This described an ancillary order encounter involving a routine screening mammogram. The imaging noted no significant findings, though the dense breast tissues reduced the sensitivity of mammography. The report confirmed no evidence of malignancy, and follow-up procedures emphasized high-risk conditions, assuring comprehensive health screening■4:15†source■. 8. **07/11/2022**: An optometry office visit occurred for the assessment of myopia, astigmatism, and presbyopia. Adjustments to spectacle prescriptions were made. Trial lenses for contact lenses were provided to assess fit and efficacy, and recommendations for future follow-ups were given based on the patient's vision assessment■4:19†source■. 9. **07/15/2024**: During a phone consultation with occupational health services, the patient expressed concern about work restrictions impacting her lunch time. Work notes were updated and communication was made transparent via kp.org. The patient felt informed and had their questions answered satisfactorily■4:18†source■. 10. **08/26/2024**: An optometry visit highlighted a need for updated glasses due to far-sighted blurriness. There was a noted family history of glaucoma. The visual exam results led to a discussion about prescription updates, family history considerations, and noted improvements in visual acuity post-refraction■4:16†source■. 11. **08/28/2024**: In the context of occupational health services, the patient's follow-up call noted a request for modified duty extension due to a clinic cancellation. There was concern over work status regarding right shoulder and elbow injuries. Documentation indicated management of occupational health protocols and adherence to needed breaks and rest■4:9†source■. Please note that these summaries focus on the provided information within the context of healthcare encounters, as reflected in the notes.

20. Diagnostic impressions

a.

right shoulder blade:

The diagnostic impressions related to the right shoulder blade identified in the medical records include: 1. **No acute fracture or significant joint disease**: Imaging reports indicate there are no acute fractures, normal alignment, and no significant joint disease or soft tissue abnormalities noted in several examinations■4:1†source■■4:3†source■. 2. **Tendinitis of Right Shoulder (M75.81)**: There are multiple mentions of right shoulder tendinitis as a diagnosis related to the pain in the shoulder area■4:1†source■■4:9†source■. 3. **Cervical Myofascial Pain Syndrome (M79.1)**: This diagnosis is associated with symptoms involving the right shoulder and neck region■4:7†source■■4:10†source■. 4. **Right Scapulalgia (M89.8X1)**: This diagnosis specifically refers to pain in the shoulder blade■4:15†source■. These findings attribute the discomfort in the right shoulder blade to a combination of tendinitis, cervical myofascial pain syndrome, and specific scapular pain conditions.

right scapula:

The diagnostic impressions related to the right scapula from the medical records are as follows: - **Right Trapezius Strain**: This is indicated by the diagnosis code S46.811A as a new condition, and is later referred to as a subsequent condition with the code S46.811D■4:0†source■. - **Right Scapulalgia**: Indicated by the diagnosis code M89.8X1■4:0†source■■4:1†source■. These conditions are identified without evidence of chemical or toxic compound involvement, and are associated with reports of right shoulder and upper trapezius discomfort■4:18†source■■4:14†source■. The clinical history indicates repetitive movements and ergonomic issues at the workplace potentially contributing to these conditions■4:17†source■.

right shoulder:

The diagnostic impressions for the right shoulder, as identified in the medical records, include the following findings and conditions: 1. **Tendinitis of Right Shoulder** - The condition is identified using the code M75.81 in the medical records. This was noted as part of the visit diagnoses■4:3†source■■4:9†source■■4:14†source■. 2. **Right Shoulder Imaging Results** - Multiple X-rays of the right shoulder showed no acute fracture, normal alignment, no significant joint disease, and no significant soft tissue abnormality■4:0†source■■4:1†source■■4:5†source■■4:10†source■. 3. **Right Shoulder Impingement** - This was noted as resolved in the assessments after history and physical examination■4:12†source■. These diagnostic impressions reflect the clinical observations and radiological findings documented during the patient’s visits for right shoulder evaluation.

right elbow:

The diagnostic impressions for the right elbow as identified from the medical records are: 1. **Right Elbow Joint Pain** - This diagnosis is noted multiple times in the records with an ICD-10-CM code M25.521■4:0†source■■4:6†source■. 2. **Right Lateral Epicondylitis (Tennis Elbow)** - This is mentioned as the primary diagnosis and is associated with symptoms like pain and swelling at the lateral epicondyle of the elbow, with the ICD-10-CM code M77.11. There is also a mention of ongoing tennis elbow not responding to conservative therapies■4:5†source■■4:13†source■■4:9†source■■4:16†source■. 3. **Right Elbow Cubital Tunnel Syndrome** - This condition is described secondary to a subluxing ulnar nerve, causing radiating symptoms into the ring and small fingers■4:11†source■. The records provide a comprehensive view of the patient's right elbow conditions, including the associated diagnostic impressions and corresponding treatments such as cortisone injections and the use of a brace.

Right Elbow:
The diagnostic impressions and findings for the Right Elbow from the medical records are as follows: 1. **Right Elbow Joint Pain**: The initial diagnostic code is M25.521, indicating right elbow joint pain. The medical notes suggest that this diagnosis is linked to a condition likely related to tennis elbow■4:0†source■. 2. **No Acute Fracture Identified**: An imaging study of the right elbow reported no acute fracture, normal alignment, and no significant joint disease or soft tissue abnormality■4:0†source■■4:1†source■■4:3†source■■4:13†source■. 3. **Right Lateral Epicondylitis**: This condition, also known as tennis elbow, was diagnosed, with symptoms including pain at the elbow improved slightly after a cortisone injection■4:5†source■■4:9†source■■4:12†source■■4:16†source■■4:18†source■. 4. **Cubital Tunnel Syndrome**: A diagnosis of right elbow cubital tunnel syndrome secondary to subluxing ulnar nerve was made. This condition was associated with compensatory shoulder pain■4:10†source■. Thus, the main diagnostic impressions for the right elbow include right elbow joint pain likely due to lateral epicondylitis (tennis elbow), no acute injury or significant structural abnormalities from imaging, and associated cubital tunnel syndrome.

Right Scapula:
The diagnostic impressions for the Right Scapula in the provided medical records are identified as: 1. Right Scapulalgia [M89.8X1]■4:4†source■. 2. There were no acute fractures, abnormal alignments, significant joint diseases, or soft tissue abnormalities identified in the X-ray evaluations■4:3†source■.

21. Causation

a.

The patient's right shoulder injury is considered work-related based on several factors documented in the medical notes. Here is a detailed evaluation: 1. **Mechanism of Injury**: The injury is described as a right shoulder sprain/strain. The patient, Michelle E. Tiu, reported developing right shoulder pain due to repetitive work activities, including reaching across her work station to turn off a light approximately 60 times daily. Additionally, she had to swing a machine overhead, which increased her right shoulder pain■4:0†source■. 2. **Ergonomic Concerns**: The patient's work environment was not ergonomically supportive, which contributed to her shoulder pain. The report notes that several colleagues were experiencing similar shoulder issues, prompting ergonomic workstation evaluations at their workplace■4:0†source■. 3. **Gradual Onset**: The symptoms developed gradually, consistent with the mechanism of repeated strain from the work-related tasks described■4:0†source■. 4. **Consistency with Work Demands**: As an optometrist, her job involves repetitive motion tasks and overhead reaching, which aligns with the described mechanism of her shoulder injury. This supports the causation link between her job tasks and the injury■4:0†source■. 5. **Medical Evaluation**: The clinical history and medical evaluations are consistent with a work-related origin of her condition. There is an emphasis on repetitive work actions contributing directly to the shoulder pain■4:0†source■■4:1†source■. These findings indicate that Michelle E. Tiu's shoulder injury is indeed related to her work activities, primarily due to repetitive strain from tasks intrinsic to her role and the non-ergonomic setup in various work locations■4:0†source■■4:1†source■.

b.

The patient's injury appears to be secondary to cumulative trauma at work due to several factors described in the medical records: 1. **Mechanism of Injury**: The patient developed right shoulder pain while performing repetitive tasks at work. She reports repetitive reaching across her workstation, turning off lights up to 60 times daily, and swinging machines overhead, which increase her right shoulder pain■4:12†source■. This kind of repetitive motion is consistent with cumulative trauma disorders. 2. **Work Environment and Ergonomics**: During the COVID-19 pandemic, the patient was required to work in multiple non-ergonomic locations, in contrast to her usual ergonomic setup. These suboptimal setups, such as improperly adjusted chairs and monitors, contributed to her shoulder pain■4:0†source■. Additionally, her work involved frequent overhead reaching and outstretched arm positioning during constant keyboarding, which led to increased right shoulder pain■4:7†source■■4:9†source■. 3. **Clinical Corroboration**: It has been documented that other colleagues also experienced similar issues, indicating a possible work-related environment concern. Ergonomic assessments have been requested, which reinforced the link to workplace conditions■4:0†source■■4:10†source■. 4. **Gradual Onset of Symptoms**: The patient reported that her symptoms started gradually, typical of cumulative trauma, as opposed to a single acute traumatic event■4:0†source■■4:2†source■. These pieces of evidence collectively support the patient's claim that her shoulder injury is secondary to cumulative trauma at work. The consistent relation of symptoms with non-ergonomic conditions and repetitive actions at her workplace corroborate the cumulative trauma diagnosis.

22. Impairment

a.

right shoulder blade:
In the AMA Guides to the Evaluation of Permanent Impairment, 5th Edition, impairment ratings for the upper extremities, including the shoulder, are located in Chapter 16. Specific impairment ratings and detailed instructions about assessing the shoulder, including the shoulder blade, involve evaluating range of motion, muscle strength, and possible neurological deficits. In this particular document, it is noted that, when calculating the impairment of the upper extremities, figures and tables such as 16-1a, 16-1b, and tables like 16-3 are utilized. The document states no specific regional impairment rating for the right shoulder based upon range of motion■4:1†source■. Furthermore, the document mentions that conditions such as tendinitis, despite being persistent, do not typically justify a permanent impairment rating unless there are extenuating factors considered■4:2†source■. The process involves assessing range of motion for various shoulder movements, including flexion, abduction, adduction, and rotations. It specifies no impairment due to peripheral nerve or vascular issues in the right upper extremity■4:2†source■. According to the AMA Guides, as outlined here, a lack of significant physical findings or limitations might not result in a ratable impairment, particularly if range of motion and functional capacity are normal■4:9†source■. Overall, the presence or absence of impairment is determined through a combination of physical examinations and how these findings correlate with standardized clinical guidelines outlined in the AMA Guides. It emphasizes the need for documented criteria such as motor deficits or limited range to justify an impairment rating■4:2†source■.

right scapula:
The AMA Guides to the Evaluation of Permanent Impairment, 5th Edition, does not specifically assign an impairment rating directly for the scapula, but rather considers the function and range of motion deficits that may be caused by conditions involving the upper extremity, including the shoulder girdle. In Chapter 16 of the AMA Guides, impairment for the upper extremity usually considers range of motion deficits. However, for certain syndromes such as tendinitis or fasciitis, the guides specify that these conditions do not warrant a permanent impairment rating unless there are other factors complicating the condition, such as significant loss of function or other measurable deficits■4:6†source■. For shoulder or scapular conditions, the impairment rating process typically involves assessing the loss in range of motion, which can be particular to flexion, extension, abduction, adduction, etc. In the case explored in your document, shoulder-related impairments have been evaluated based on these parameters, but the exact percentage allocated to a right scapula condition was not explicitly defined within the document itself■4:6†source■. The impairment ratings are intended to gauge the extent to which a functional limitation impacts daily living activities and employability, usually as a result of an industrial injury. This is particularly relevant in worker's compensation cases where such ratings are instrumental for compensation deliberations■4:6†source■. Thus, to determine an impairment related to the right scapula using the AMA Guides, a detailed functional capacity assessment and range of motion evaluation would be required, as defined by the criteria in the AMA Guides, 5th Edition■4:6†source■.

right shoulder:
Based on the search results from the AMA Guides, 5th edition, the right shoulder's impairment rating is evaluated primarily through its range of motion as outlined in Chapter 16. The AMA Guides consider various factors such as loss of range of motion, strength, and structural integrity to determine impairment ratings. For the right shoulder, the impairment rating process involves: 1. **Range of Motion Measurements**: The active range of motion of flexion was measured at 170 degrees, abduction at 170 degrees, external rotation at 60 degrees, and internal rotation to the T6 vertebra level. These measurements are assessed against normal values, and any deviations contribute to the impairment rating■4:0†source■. 2. **Special Tests**: The presence of a positive Hawkins test and absence of other signs such as Neers or painful arc help in assessing shoulder impingement or other possible conditions■4:7†source■■4:14†source■. 3. **Nerve Function and Sensation**: An evaluation of normal sensation and normal deep tendon reflexes (DTRs) indicates no additional impairment due to neurological factors■4:7†source■. 4. **Discharge Notes**: A note on discharge stated that the patient was released back to work without restrictions and a consideration was made per the AMA guides that no ratable impairment was noted since conditions were resolved■4:0†source■■4:3†source■. 5. **Diagnoses and Treatment**: Consistent with the guidelines, tendinitis of the shoulder and periods of improved/corrected shoulder impingement are considered when assigning impairment ratings. No permanent impairment may be assigned if conditions have resolved or improved substantially per AMA guidelines■4:17†source■. The AMA Guides emphasize that many conditions, despite being chronic or recurrent, do not necessarily translate into permanent impairment unless there are persistent deficits that affect the range of motion, strength or neurological function. In this case, despite postings indicating previous shoulder conditions, the improvements noted in physical assessments led to no permanent impairment rating■4:3†source■.

right elbow:
The impairment rating for the right elbow using the AMA Guides, 5th edition, would consider several factors such as range of motion, strength, and nerve function. Here is a detailed explanation based on the guidelines: 1. **Range of Motion**: For the AMA Guides, particularly the 5th edition, the range of motion is a critical factor in determining the impairment rating for the upper extremity, including the elbow. Specific measurements from Figures 16-40, 16-43, and 16-46 guide this assessment. Ratings are assigned based on how much the range of motion deviates from normal values. 2. **Peripheral Nerve System**: If there is any decrease in function of the peripheral nerve, it would also impact the impairment rating. However, according to the document, there is no peripheral nerve system

impairment noted for the right upper extremity■4:0†source■. 3. **Inflammatory Conditions**: Chronic conditions like tendinitis, fasciitis, or epicondylitis are considered separately. According to Section 16.7d of the AMA Guides, these conditions do not contribute to a permanent impairment rating unless additional permanent factors are present■4:1†source■. 4. **Grip Strength**: There is often a typical assessment of grip strength using devices like the JAMAR dynamometer. However, in the context of the provided document, there was no grip strength deficit noted, nor was one anticipated■4:1†source■. Based on these evaluation metrics, there would need to be specific deviations or impairments noted in these areas to substantiate an impairment rating. From the provided information, unless complications or other permanent factors are present, conditions might not receive a permanent impairment rating. Without these, the impairment rating would likely be low or non-existent for the right elbow if following the AMA 5th edition strictly as outlined■4:0†source■■4:1†source■.

Right Elbow:
The AMA Guides to the Evaluation of Permanent Impairment, 5th Edition, provides specific guidelines for evaluating upper extremity impairments, including the right elbow. The impairment rating for the right elbow is determined by evaluating the range of motion and any associated conditions like tendinitis or epicondylitis. In the 5th edition, conditions such as tendinitis or epicondylitis are given impairment ratings primarily when there are additional factors to consider. The guidelines specify that, although these conditions may be persistent, they are not typically given a permanent impairment rating unless there's another significant factor involved■4:0†tmpyms8hkgn.pdf■. For the evaluation, the range of motion tests and grip strength assessments are crucial determinants. The ratings often convert regional impairments into whole-person impairment percentages using specific tables and figures from Chapter 16. For example, impairments of the upper extremity might be assessed as regional impairments and then calculated to provide a percentage of whole-person impairment using conversion tables such as Table 16-3■4:0†tmpyms8hkgn.pdf■. Overall, this thorough assessment helps correlate the functional limitations caused by the condition to an impairment rating that is considered in determining disability compensation or needed adjustments in the workplace. If you need additional information on specific figures or tables like Figure 16-40, Figure 16-46, or others referenced for precise measurements and calculations, those should be consulted directly from the AMA Guides documentation.

Right Scapula:
From the documents, the impairment rating for the right scapula as per the AMA Guides, 5th Edition, involves various factors primarily based on the functional limitations and clinical findings associated with shoulder conditions. However, specific details for only the scapula were not retrieved directly in the text. The AMA Guides typically consider upper extremity as a whole when assigning impairment ratings, catching conditions that might affect shoulder, arm, and the area around the scapula. ### Detailed Description: 1. **Functional Impairment**: - The primary focus is on the range of motion limitations, strength deficiencies, and pain impact associated with conditions involving the scapula■4:0†source■. 2. **Figures and Tables**: - Typically, figures like Table 16-3 are used in the AMA Guides to convert regional impairments (found via tables for specific joint movements) into whole-person impairment ratings■4:0†source■. For shoulder-related impairments, figures such as 16-40, 16-43 are often used, which list the specific percentages of impairment based on reduced range of motion■4:3†source■. 3. **Consideration of Medical Conditions**: - According to the AMA 5th edition guidelines, conditions such as tendinitis, bursitis, or other musculoskeletal conditions might not be assigned a permanent impairment rating unless they significantly affect function or present additional concerns■4:0†source■. 4. **Clinical Evaluation**: - Findings on the function and structural integrity of shoulder components are essential. Conditions like impingement syndrome, tendinitis, and myofascial pain syndrome around the scapula can contribute to the impairment rating when they are chronic and affect the patient's ability to perform daily tasks■4:8†source■■4:12†source■. This description indicates the importance of assessing combined elements from functional capacity, clinical diagnosis, and the specific evaluation figures and tables outlined in the AMA Guides for determining the exact impairment rating for right scapular and shoulder conditions. Direct quotes were not found for the scapula alone, suggesting the rating process integrates components of shoulder assessments

comprehensively.

23. Periods Of Total Disability

a.

right shoulder blade:
The average duration off work due to a right shoulder blade injury, followed by a transition to light duty, is typically determined by the severity of the injury and the specific medical recommendations. In the case provided, the patient was placed off work from 06/17/2024 through 06/25/2024 due to an incapacitating injury or pain. Following this period, the patient was transitioned to modified activity at work and at home from 06/26/2024 through 07/09/2024■4:1†source■. Similarly, in another instance from 2016, a patient with a similar shoulder injury was on modified activity and returned to usual tasks after about a month of graded return to work■4:2†source■. Therefore, in this example, the total time before transitioning to light duty was approximately 9 days off work before modified or light duty was recommended, though variations can occur based on the nature of the injury and recovery progress.

right scapula:
The documents indicate that for a right scapula injury, individuals may be placed off work due to incapacitating injury or pain from a specified period, and then transitioned to modified activity. For example, one case mentioned involved being off work from 06/17/2024 through 06/25/2024 and then transitioning to modified activity from 06/26/2024 through 07/09/2024■4:0†source■. This implies an average of about 1 week off work before transitioning to light duty, assuming the condition permits modified work activities. However, this can vary based on the severity of the injury and employer accommodations. The modified activity might include specific conditions such as requiring breaks after a certain duration of work■4:0†source■. The transition to light duty often includes restrictions like limited lifting and specified working hours■4:1†source■. Always refer to specific professional recommendations and employer policies.

right shoulder:
The documents indicate that, in the context of shoulder injuries such as right shoulder impingement, the transition to modified or light duty is determined by the treating physician. In a specific case depicted in the documents, the patient, Michelle E. Tiu, was placed on modified activity at work and at home for about four weeks. During this period, her modified work status included restrictions like not reaching above the right shoulder and limitations on lifting/carrying/pushing/pulling to no more than 5 pounds. Initially, she worked half the day on full duty and the other half on modified duty. This gradually increased to full duties over several weeks based on the improvement of her condition■4:0†tmpyms8hkgn.pdf■. Therefore, for similar shoulder injuries, the time taken to transition from complete rest to light duty varies depending on the severity of the injury and the individual’s recovery progress, under medical supervision. This specific case indicates a gradual increase back to full duties occurring over approximately four weeks, but such timelines can differ between cases.

right elbow:
The documents provide some information regarding the transition from off work to light-duty work for a right elbow injury. Based on the clinical notes, it appears that the patient was placed off work due to an incapacitating injury or pain from 06/17/2024 through 06/25/2024, and then transitioned to a modified activity status from 06/26/2024 through 07/09/2024■4:0†source■. Another document indicates a period of modified activity continuing from 07/31/2024 through 09/05/2024■4:5†source■. Typically, these modified duty periods suggest a gradual return to work responsibilities, depending on the

patient's condition and the ability of the employer to accommodate these restrictions. Please note that this specific timeline might vary based on individual circumstances, employer policies, or further medical evaluations. The provided details suggest an initial off-work period of almost one week followed by a two-week light-duty period, which could serve as a benchmark for similar injuries.

Right Elbow:
The documents contain information about occupational injury handling, but they do not provide a specific average duration that an American would be off work for a right elbow injury before transitioning to light duty. It appears that the decision for transitioning to light duty is highly individualized based on the severity of the injury, treatment progress, and accommodations available. For instance, in the case of Michelle E. Tiu, after a right elbow and shoulder injury, she was placed off work from 6/17/2024 through 6/25/2024. She then transitioned to modified activity from 6/26/2024 through 7/9/2024, with a recommendation for breaks during the workday■4:1†source■. Thus, in this example, the transition to modified duty occurred approximately 9 days after being completely off work, but it required specific work accommodations, such as regular breaks■4:0†source■■4:4†source■. This suggests that while there is no standard timeline universally applied, the transition to light duty can begin within a few weeks of the initial injury, depending on the individual's recovery and workplace accommodations available.

Right Scapula:
For an injury related to the right scapula, the transition to light duty after being completely off work can vary based on individual cases and treatment progression. However, according to the document, a patient with a right scapula injury was placed off work starting from 06/17/2024 through 06/25/2024 and was then transitioned to modified (or light) duty from 06/26/2024 through 07/09/2024■4:0†source■. This suggests that in this specific case, after approximately a little over a week of being off work completely, the patient was transitioned to modified duty, contingent on the employer being able to accommodate the modified activity. If the modifications cannot be accommodated, the patient would continue to be considered temporarily and totally disabled from their regular work■4:1†source■. The requirements for modified duty included a need for a 20-minute break from activity after every 20 minutes of work activity■4:0†source■. It is important to note that each case may be different based on the severity of the injury and individual recovery progress.

24. Future Medical Care

a.

right shoulder blade:
According to the MTUS (Medical Treatment Utilization Schedule) guidelines, for tendinitis of the right shoulder and associated conditions like cervical myofascial pain syndrome, future medical care options recommended include: 1. **Acupuncture**: This is considered when pain medication is reduced or not well tolerated. It serves as an adjunct to physical rehabilitation and/or surgical intervention to hasten functional recovery. Acupuncture may help reduce pain, inflammation, increase blood flow, range of motion, relieve medication-induced nausea, promote relaxation, and minimize muscle spasms■4:0†source■. 2. **Physical and Occupational Therapy**: The guidelines recommend physical therapy for range of motion exercises, modalities, and therapeutic exercises aimed at relieving symptoms and preventing future exacerbations. The suggested frequency is 1-2 times a week for 3-6 weeks, especially as part of a comprehensive approach that may include acupuncture■4:4†source■■4:5†source■. 3. **Pain Management**: Non-steroidal anti-inflammatory drugs (NSAIDs) like Ibuprofen can be prescribed with monitoring, although the use of such medications

should be balanced with other treatments like acupuncture to reduce reliance■4:4†source■. These treatments are part of an integrated approach to manage symptoms and prevent future exacerbations, consistent with MTUS and ODG (Official Disability Guidelines) recommendations for managing tendinitis and related conditions.

right scapula:
According to the MTUS (Medical Treatment Utilization Schedule) and the ODG (Official Disability Guidelines) referenced in the provided documents, the following future medical care is recommended for addressing symptoms related to the right scapula, such as right scapulalgia: 1. **Physical Therapy**: - Patients with right scapulalgia are recommended to continue additional physical therapy, focusing on myofascial release, scapular stabilizers, correcting upper trapezius recruitment, and establishing a home exercise program. This aligns with the guidelines for physical medicine treatment within the MTUS for chronic pain■4:0†source■. - Physical therapy is suggested once a week for six weeks, totaling six treatments. It was noted that previous physical therapy had reduced pain by 50%, indicating its efficacy in pain management and improving function in activities of daily living (ADLs)■4:0†source■. 2. **Ergonomic Evaluation**: - An ergonomic evaluation is requested to assess workplace modifications that could reduce aggravating factors, improve function, and facilitate return to work. This approach helps to prevent future exacerbations by correcting the environment contributing to the symptoms■4:0†source■■4:18†source■. 3. **Acupuncture**: - Acupuncture is recommended as an adjunct treatment to manage pain and spasm, especially when pain medications are not well tolerated. This is per MTUS Acupuncture Medical Treatment Guidelines, which suggest acupuncture can hasten functional recovery and alleviate pain■4:6†source■. 4. **Medication Management**: - Continued use of medications like ibuprofen is recommended as needed for pain. It's advised to discontinue if certain side effects, such as stomach upset, nausea, or rashes, develop■4:10†source■■4:19†source■. 5. **Imaging and Further Diagnostic Evaluations**: - If symptoms persist, repeat and/or advanced imaging and other investigations may be beneficial to assess any underlying conditions or physical changes that might require other interventions■4:17†source■. These interventions are oriented towards alleviating current symptoms and preventing future exacerbations by addressing both the physiological and environmental factors associated with the patient's condition.

right shoulder:
According to the MTUS (Medical Treatment Utilization Schedule) and ODG (Official Disability Guidelines) guidelines, future medical care for the right shoulder, specifically addressing tendinitis or similar musculoskeletal issues, can include the following treatments aimed at curing or relieving symptoms or preventing future exacerbations: 1. **Acupuncture**: MTUS guidelines suggest acupuncture as an option when pain medication is reduced or not tolerated. It can be used as an adjunct to physical rehabilitation and surgical intervention, playing roles in pain reduction, inflammation control, blood flow improvement, range of motion increase, and muscle spasm reduction■4:0†source■. 2. **Physical Therapy**: Recommended to improve function, increase strength, range of motion, and flexibility, thereby minimizing discomfort. MTUS Physical Medicine Treatment Guidelines underline physical therapy as suitable for chronic pain management. It should be done at a frequency of 1-2 times a week for a duration of 3-6 weeks, focusing on range of motion, joint mobilizations/manual therapy, modalities, and establishing a home exercise program■4:3†source■■4:2†source■. 3. **Ergonomic Assessment and Interventions**: Ergonomic assessments of workstations are necessary, especially if the shoulder pain originated from repetitive tasks or poor ergonomics at work. Recommendations for workstation modifications should be implemented to facilitate patient recovery and prevent further injury■4:19†source■. 4. **Medication**: Non-steroidal anti-inflammatory drugs (NSAIDs) such as ibuprofen can be continued with food as needed to manage pain, provided there are no adverse effects such as stomach upset■4:19†source■. 5. **Advanced Imaging**: If symptoms persist despite conservative management, repeat or advanced imaging and other investigations might be helpful to further assess the underlying issues■4:8†source■. 6. **Repeat Trapezius Trigger Point Injections**: If conservative measures are not sufficiently effective, trigger point injections might be considered for managing pain and improving function■4:13†source■. These interventions are

designed to relieve discomfort, enhance functional recovery, and prevent future exacerbations. They should be tailored to the individual's specific symptoms and response to initial treatment.

right elbow:

To address future medical care needs for right elbow conditions according to the Medical Treatment Utilization Schedule (MTUS) and Official Disability Guidelines (ODG), several approaches are suggested. 1. **Physical Therapy**: Continued physical therapy is recommended to help improve functionality, increase strength, range of motion, and overall minimize discomfort, which is appropriate per MTUS and includes specific guidelines for chronic pain management through physical medicine treatments■4:16†source■. 2. **Acupuncture**: This treatment is recognized under MTUS guidelines as a modality when pain medication is reduced or is not tolerated. Acupuncture is used adjunctively to physical rehabilitation and/or surgical interventions, and is intended to hasten functional recovery. This therapy can also reduce pain, inflammation, increase range of motion, and promote relaxation■4:0†source■■4:9†source■. 3. **Ergonomic Evaluation**: For workplace modifications to help reduce aggravating factors and improve function. It is a pertinent preventative strategy suggested under MTUS guidelines to mitigate exacerbations■4:18†source■. 4. **Injections and Medications**: Use of Platelet Rich Plasma (PRP) injections and other corticosteroid forms can be part of the therapeutic regime as mentioned for cases of lateral epicondylitis and is utilized for continued relief or management■4:11†source■. 5. **Home Exercise Program**: Including routines focusing on strengthening, scapular stabilization, and prescribing the use of assistive devices such as a tennis elbow brace. Detailed exercise instructions, like wrist flexor and extensor stretch, are provided to patients for self-management■4:10†source■■4:19†source■. Each of these treatments is chosen based on patient-specific factors and insurer provisions, balancing conservative approaches with more interventional procedures as guided by condition severity and response to initial treatments.

Right Elbow:

According to the MTUS (Medical Treatment Utilization Schedule) and ODG (Official Disability Guidelines), the following future medical care is recommended for managing right elbow conditions and preventing future exacerbations: 1. **Physical Therapy**: It is referenced as appropriate per MTUS Chronic Pain Medical Treatment guidelines. Typically, physical therapy is recommended at a frequency of 1-2 times a week for 3-6 weeks, totaling around 6 treatments. It has shown to improve functions in activities of daily living (ADLs), decrease pain, reduce medication usage, and decrease work restrictions■4:16†source■. 2. **Acupuncture**: As per the MTUS Acupuncture Medical Treatment guidelines, acupuncture is an option particularly when pain medication is reduced or not tolerated. It can be used as an adjunct to physical rehabilitation to hasten functional recovery. The frequency recommended is 1-2 times a week for 3-6 weeks, with a total of about 6 treatments■4:1†source■. 3. **Occupational Therapy/Ergonomics Evaluation**: An ergonomic evaluation is suggested to assess workplace modifications to help reduce painful stimuli and improve function. This aligns with ensuring that any workplace aggravating factors are addressed■4:5†source■. 4. **Medications/Conservative management**: Continuation of current pain management strategies like NSAIDs may be advised if well-tolerated, ensuring they do not exceed recommended dosages■4:9†source■. Clinical guidelines recommend the use of these interventions to maintain or improve function and control symptoms for conditions like right elbow pain or lateral epicondylitis, thus helping with recovery and return to work■4:10†source■.

Right Scapula:

According to the MTUS (Medical Treatment Utilization Schedule) and ODG (Official Disability Guidelines) regarding future medical care for a condition like "Right Scapulalgia": 1. **Physical Therapy**: - Additional physical therapy is recommended to be 1 time a week for 6 weeks. This has been reported to decrease pain by 50%, improve function in activities of daily living (ADLs), reduce medication usage, and ease work restrictions■4:0†source■. 2. **Ergonomic Evaluations**: - An ergonomic evaluation is suggested to assess and possibly modify the workplace environment to help reduce factors that aggravate the condition and improve function, promoting a return to work■4:0†source■. 3. **Acupuncture**: - Acupuncture is considered as an adjunctive treatment option when pain medication is reduced or not tolerated. It can help in reducing pain, inflammation, increasing range of motion, and promoting relaxation■4:6†source■. These interventions align with the Chronic Pain Medical Treatment guidelines under MTUS and aim to relieve current symptoms and prevent future exacerbations.